Nineteenth Meeting of the WHO South-East Asia Regional Certification Commission for Poliomyelitis Eradication

Colombo, Sri Lanka, 24-26 June 2026

Report of the Meeting

World Health Organization
REGIONAL OFFICE FOR South-East Asia

Nineteenth Meeting of the WHO South-East Asia Regional Certification Commission for Poliomyelitis Eradication
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Abbreviations and acronyms

AFP	acute flaccid paralysis
bOPV	bivalent oral poliovirus vaccine
cVDPV	circulating vaccine-derived poliovirus
EPI	Expanded Programme on Immunization
ES	environmental surveillance
GAPIV	Global Action Plan for Poliovirus Containment, fourth edition
GCC	Global Commission for the Certification of the Eradication of Poliomyelitis
GIN	Global Immunization Newsletter
GPEI	Global Polio Eradication Initiative
IPV	inactivated poliovirus vaccine
ITAG	Immunization Technical Advisory Group
IVD	Immunization and Vaccine Development
NAC	National Authority for Containment
NCCPE	National Certification Committee for Poliomyelitis Eradication
NCTF	National Containment Task Force
NITAG	National Immunization Technical Advisory Group
OPV	oral poliovirus vaccine
PEF	poliovirus-essential facility
PID	primary immunodeficiency disorder
SEA-RCCPE	South-East Asia Regional Certification Commission for Poliomyelitis Eradication

SEARO	WHO Regional Office for South-East Asia
SIA	supplementary immunization activity
VDPV	vaccine-derived poliovirus
VPD	vaccine-preventable disease
WHO	World Health Organization
WPV	wild poliovirus

Executive summary

The Nineteenth Meeting of the WHO South-East Asia Regional Certification Commission for Poliomyelitis Eradication (SEA-RCCPE) was held in Colombo, Sri Lanka, from 24 to 26 June 2026. The Commission reviewed annual reports, for the year 2025, from National Certification Committees for Poliomyelitis Eradication (NCCPEs), country presentations and regional programme data to assess progress in sustaining the Region's polio-free status and to review developments related to population immunity, surveillance, outbreak preparedness and response, poliovirus containment, and polio transition.

The Commission concluded that the WHO South-East Asia Region continues to maintain its wild poliovirus-free status. It acknowledged the sustained commitment of Member States, NCCPEs, surveillance networks, laboratories and partners in protecting this achievement and maintaining strong programme performance across the Region.

The Commission noted that regional population immunity remains generally high and that surveillance systems continue to provide a strong platform for the early detection of poliovirus transmission. However, important immunity gaps persist among zero-dose and under-immunized populations, hard-to-reach communities, migrant and displaced populations, and areas with long-standing subnational disparities in vaccination coverage. The Commission further noted that three countries in the Region have not yet introduced a second dose of inactivated poliovirus vaccine (IPV2), which may lead to suboptimal population immunity against poliovirus type 2. The Commission also noted continuing operational challenges in surveillance, including variations in AFP detection, stool adequacy, timeliness of reporting and specimen transport in some countries.

The meeting reviewed recent variant poliovirus detections in the Region and noted that all events were identified through sensitive surveillance systems and followed by timely investigations and response measures. While these responses demonstrated the effectiveness of existing surveillance and response mechanisms, they also highlighted the need for continued vigilance and sustained programme performance.

The Commission further reviewed progress in poliovirus containment and polio transition. While containment activities are generally progressing well across the Region, country-specific gaps remain, including the need for updated facility inventories, strengthened containment structures and continued compliance with GAPIV requirements. Progress in transitioning essential polio functions into national health systems was also reviewed. Although some advances have been made, transition milestones remain behind schedule in several priority countries, raising concerns about the long-term sustainability of surveillance, laboratory and outbreak response capacities in the context of declining external financing and the increasing risk of losing trained and experienced human resources before their functions are fully institutionalized within national health systems.

The Commission recommended that Member States, with the continued support of partners and other stakeholders, continue to strengthen routine immunization and surveillance, particularly at subnational level; review and optimize polio vaccine schedules in line with WHO and SAGE

recommendations; strengthen outbreak preparedness and risk assessment; enhance cross-border collaboration and data sharing; optimize environmental surveillance based on epidemiological risk; strengthen containment oversight; and secure sustainable domestic financing to maintain essential polio functions over the long term and support effective transition planning.

Country-specific conclusions and recommendations were also provided for all countries. Collectively, these recommendations aim to address remaining challenges in improving population immunity, surveillance, containment and transition while safeguarding the Region's polio-free status and supporting broader regional immunization and health security goals.

The SEA-RCCPE concluded that sustaining a polio-free South-East Asia Region will require continued political commitment, strong immunization and surveillance systems, effective containment, and successful integration of essential polio functions into resilient national health systems.

1. Introduction

The WHO South-East Asia Region was certified polio-free by the South-East Asia Regional Certification Commission for Poliomyelitis Eradication (SEA-RCCPE) in March 2014 and has since maintained its status as free of indigenous wild poliovirus transmission. Sustaining this achievement remains a major public health priority for Member States, particularly in the context of continued wild poliovirus transmission in endemic countries, ongoing outbreaks of circulating vaccine-derived polioviruses (cVDPVs) in several regions, increasing population mobility, humanitarian emergencies, and persistent immunity gaps among vulnerable populations. The maintenance of polio-free status therefore requires sustained high population immunity, sensitive surveillance systems, effective outbreak preparedness and response, and rigorous implementation of poliovirus containment measures.

The global Polio Eradication Strategy (2022–2029) continues to provide the overarching framework for achieving and sustaining global polio eradication. As the global programme advances towards eradication and eventual cessation of oral polio vaccines, countries are increasingly required to strengthen preparedness for the post-eradication era. This includes maintaining highly sensitive surveillance systems capable of rapidly detecting any poliovirus event, implementing robust outbreak response mechanisms, and ensuring full compliance with the Global Action Plan for Poliovirus Containment (GAPIV). Consequently, containment has become an essential component of certification activities and a critical requirement for safeguarding the gains achieved through polio eradication.

In parallel, countries in the Region are navigating the transition of polio assets and functions as external financing for polio eradication gradually declines. The polio transition agenda seeks to ensure that critical capacities developed through the eradication programme—particularly surveillance, laboratory networks, outbreak preparedness and response, data systems, workforce capacity and community engagement mechanisms—are sustainably integrated into national health systems. Successful transition is essential not only for maintaining a polio-free Region but also for protecting broader health security gains and supporting the delivery of other public health priorities.

The Regional Vaccine Implementation Plan (RVIP), which serves as the regional framework for implementing the goals of Immunization Agenda 2030 (IA2030), provides an important framework for sustaining and maximizing the benefits of investments made through the polio programme. The essential capacities established through polio eradication contribute directly to IA2030 and RVIP objectives by strengthening routine immunization systems and reaching underserved populations to provide equitable access to immunization services (Strategic Priority 3: Coverage and Equity), vaccine-preventable disease surveillance, laboratory networks, outbreak preparedness(Strategic Priority 6: Outbreaks and Emergencies) , risk assessment, and data use for evidence based decision making , (Strategic Priority 7: Research, Innovation and Data).. The continuation and integration of these functions are therefore critical for achieving regional immunization goals and advancing universal access to life-saving vaccines.

The South-East Asia Region has consistently emphasized the importance of integrating polio essential functions into broader immunization and health system agendas. Surveillance systems originally established for polio are increasingly supporting integrated vaccine-preventable disease surveillance, environmental surveillance platforms are contributing to broader public health monitoring, and laboratory networks are strengthening capacity for detection of multiple pathogens. Sustaining these assets through effective transition planning is therefore closely aligned with both regional immunization priorities and wider health system strengthening efforts.

The SEA-RCCPE plays a critical role in supporting these objectives through independent oversight of regional and national efforts to maintain polio-free status. The Commission reviews annual reports submitted by National Certification Committees for Polio Eradication (NCCPEs), assesses evidence related to population immunity, surveillance quality, outbreak preparedness, containment and transition, and provides recommendations to Member States and WHO. Through this process, the Commission contributes to the dual objectives of sustaining a polio-free Region and ensuring that the investments and capacities developed through the Global Polio Eradication Initiative continue to support regional immunization goals and public health priorities. The Nineteenth Meeting of the SEA-RCCPE was convened in Colombo, Sri Lanka, from 24 to 26 June 2026.

2. Objectives of the meeting

The primary objective of the meeting was to assess progress in sustaining the wild poliovirus-free status of the South-East Asia Region and to provide the Commission with a conclusion on the regional polio-free status.

The specific objectives of the meeting are enumerated below.

- Maintain polio-free status
    - To review country performances on maintaining polio-free status, based on reports by National Certification Committees for Polio Eradication, with focus on :
        - Maintaining population immunity;
        - Polio surveillance;
        - National risk assessments;
        - Outbreak preparedness and response
        - Containment and post-certification strategy
- Post-certification strategy:
    - To review and provide inputs to the post-certification strategy, including containment of polioviruses
- Polio Transition
    - To review progress towards Polio transition
- Accountability

- To review the implementation status of recommendations made at the 18th meeting of the SEA-RCCPE.

3. Organization of the meeting

The meeting was inaugurated by the Deputy Minister of Health and Mass Media, Government of Sri Lanka, along with the Acting WHO Representative to Sri Lanka and the Chair of the Global Certification Commission for Poliomyelitis Eradication. The agenda included opening and closing sessions, global and regional updates, country presentations by NCCPEs, containment updates, a consultation on the polio transition, updates from the Eastern Mediterranean and Western Pacific Regions, and closed-door deliberations by SEA-RCCPE members and the Secretariat.

The meeting brought together the SEA-RCCPE Chair and members, representatives of NCCPEs, National Containment Task Forces and National Authorities for Containment, the polio laboratory network, the Ministry of Public Health of Sri Lanka as host country, partner agencies, observers, WHO headquarters, WHO regional offices and WHO country offices.

The meeting followed a three-day structure. Day 1 focused on global and regional updates and initial country reviews. Day 2 continued NCCPE reviews and reviewed containment. Day 3 focused on the polio transition, inter-regional updates, closed-door deliberations, and the presentation of conclusions and recommendations.

4. Methodology of the regional review

The regional review was based on annual progress reports for 2025 submitted by NCCPEs, country presentations during the meeting, containment updates, regional and global surveillance and programme data, and discussions among Commission members, NCCPE representatives and the Secretariat.

Each country report was allocated to two SEA-RCCPE members for detailed review. The members reviewed the reports before and during the meeting, examined key issues and clarifications, and contributed to the Commission deliberations. The reviewer allocation for the 19th meeting is summarized below.

Country	Primary reviewer	Secondary reviewer
Bangladesh	Dr Jayaprakash Muliyil	Dr Sunethra Gunasena
Bhutan	Dr Khin Nyo Thein	Dr Nobuhiko Okabe
Democratic People's Republic of Korea	Dr Nobuhiko Okabe	Dr Khin Nyo Thein
India	Dr Shrijana Shrestha	Dr Arlene King
Maldives	Dr Milena Maria	Dr Pasakorn Akarasewi
Myanmar	Dr Pasakorn Akarasewi	Dr Shrijana Shrestha
Nepal	Dr Sunethra Gunasena	Mr Chris Maher
Sri Lanka	Dr Yagob AL-Mazrou	Dr Chowdhury Ali Kawser

Thailand	Mr Chris Maher	Dr Jayaprakash Muliyil
Timor-Leste	Dr Chowdhury Ali Kawser	Dr Yagob AL-Mazrou

In addition, Ms Ellyn Ogden reviewed the transition section from the annual reports of the four transition priority countries (Bangladesh, India, Myanmar, and Nepal).

Following the country presentations and discussions, the SEA-RCCPE met in closed-door sessions to review the evidence, agree on overarching and country-specific conclusions, and formulate recommendations for Member States, NCCPEs, WHO and partners.

5. Situation update and issues reviewed

5.1 Global and regional risk context

The global update highlighted that the international spread of poliovirus continues to constitute a Public Health Emergency of International Concern, with ongoing WPV1 transmission in Afghanistan and Pakistan, continued cVDPV outbreaks across multiple regions, and poliovirus detections in countries using IPV-only schedules. Despite progress in reducing poliovirus transmission, persistent immunity gaps, uneven surveillance performance and a challenging global funding environment underscore the need to sustain eradication-level programme performance until global eradication is achieved.

The regional update emphasized that the South-East Asia Region remains polio-free; however, continued vigilance is required because of the ongoing risk of poliovirus importation, recent variant poliovirus detections, subnational immunity gaps, uneven IPV coverage, delayed introduction of a second IPV dose in three countries, and sustainability challenges associated with transition and declining external financing.

5.2 Population immunity

The regional update reviewed polio vaccine schedules across countries. All countries except Democratic People's Republic of Korea are providing both bOPV and IPV. Four countries are providing at least two doses of fractional IPV (fIPV): Bangladesh, India, Nepal and Sri Lanka. Three countries are providing two full doses of IPV: Bhutan, Myanmar and Thailand. Democratic People's Republic of Korea, Maldives and Timor-Leste were noted as countries yet to introduce IPV2.

Country	bOPV schedule	IPV/fIPV schedule
Bangladesh	6w, 10w, 14w	fIPV 14w, 9m (2026)
Bhutan	B, 6w, 10w, 14w	14w, 8m
Democratic People's Republic of Korea	NA; IPV-only schedule	14w
India	B, 6w, 10w, 14w, 16–24m	fIPV 6w, 14w, 9m
Maldives	2m, 4m, 6m	6m
Myanmar	2m, 4m, 6m	4m, 9m (from 2026)
Nepal	6w, 10w, 14w	fIPV 14w, 9m
Sri Lanka	2m, 4m, 6m, 18m	fIPV 4m, 9m (from 2026)

Thailand	6m, 1y6m, 4y; sequential schedule	2m, 4m
Timor-Leste	6w, 10w, 14w	14w

The latest WHO and UNICEF Estimates of National Immunization Coverage (WUENIC) showed the WHO South-East Asia Region as the highest performing among the six WHO regions, sustaining high routine immunization coverage across key childhood vaccines. In 2025, DTP3 and IPV1 coverages were 94% and IPV Complete Series (IPVC) coverage was 77%. While coverage data indicated strong regional performance overall, important country and subnational variations remain. An estimated 1.2 million children in the Region did not receive the DTP1 vaccine in 2025. Myanmar and Timor-Leste continued to show lower coverage than most other countries.

5.3 Surveillance and laboratory performance

Regional surveillance sensitivity was maintained, although national and subnational variations remained. Regional non-polio AFP rate was reported as 4.83 in 2025 and 2.88 in 2026, based on data available as of 22 June 2026. The percentage of adequate stool specimen collection for the Region was 90% in both 2025 and 2026. Some AFP cases were noted as pending final classification.

Environmental surveillance included 101 sites in five countries. The countries with environmental surveillance included Bangladesh, India, Myanmar, Nepal and Thailand. Environmental surveillance performance varied by country and year, and the Commission emphasized the need for feasibility assessment and optimization based on epidemiological risk.

Laboratory indicators reviewed by the meeting showed approximately 52,000 AFP specimens and approximately 2200 environmental surveillance samples tested in 2025. More than 97% of primary isolation results were available within 14 days, approximately 450 poliovirus isolates were referred for intratypic differentiation, and approximately 99% of ITD results were available within seven days.

5.4 Poliovirus containment

The containment sessions reviewed global containment strategy, national inventories, poliovirus-essential facilities, and regional containment progress. Containment implementation in the Region was assessed as mostly on track, while country-specific issues remained. The Commission emphasized that national facility inventories should be updated and that countries should establish or strengthen NCTFs or NACs as appropriate.

- Countries were reminded of the need to maintain updated inventories of facilities retaining poliovirus infectious and potentially infectious material.
- Countries with PEFs are advised to ensure certification in line with GAPIV requirements and published GCC timelines.
- Countries without updated facility inventories were encouraged to conduct surveys, especially for facilities capable of long-term storage of stool and nasopharyngeal swab specimens.

5.5 Polio transition

The meeting reviewed the regional polio transition strategy and country progress. Four countries in the Region are priority countries for polio transition: Bangladesh, India, Myanmar and Nepal. Three countries have moved from the global transition priority list to the watch list, while Myanmar remains on the priority list. The transition context is shaped by variable country readiness, declining external funding and the need to sustain essential polio functions.

The regional transition strategy uses a three-pronged approach: human resource mainstreaming, functional transition and financial sustainability. The strategy promotes phased, risk-based and country-specific pathways, recognizing differences in epidemiological, political and health system contexts across countries. The meeting emphasized that transition should not be seen as an exit strategy but as a sustainability strategy to protect polio gains and integrate essential functions into broader health systems.

The draft regional transition strategy was shared with participants, who were organized into country-specific groups and given a template to provide their inputs. All country groups submitted completed templates to the Secretariat, and these inputs will inform finalization of the strategy.

5.6 Implementation status of key recommendations from the 18th meeting

Recommendation from 18th SEA-RCCPE	Status reported at 19th meeting
Strengthen surveillance and immunization strategies at national and subnational levels, focusing on low-performing and high-risk areas.	Ongoing
Conduct annual polio outbreak simulation exercises, especially in high-risk zones.	Ongoing; conducted in Myanmar and India
Enhance cross-border collaboration for surveillance, data sharing and coordinated outbreak responses.	In progress; India-Nepal cross-border meetings conducted and information sharing with WPRO reported
Transition priority countries to convene NCCPE meetings to review transition and update national transition plans.	NCCPEs of three of four countries reported on transition; progress variable
Include updated facility inventories and outbreak preparedness plans in future NCCPE reports.	New outbreak response SOPs released on 23 June 2026
Seek SEAR-ITAG guidance on bOPV cessation and IPV-only schedules in line with SAGE recommendations.	Completed

6. Conclusions and recommendations

6.1 Overarching conclusions by SEA-RCCPE

The WHO South-East Asia Region continues to maintain its wild poliovirus-free status, despite a challenging global epidemiological environment characterized by the continued circulation of wild poliovirus type 1 in endemic countries and ongoing outbreaks of circulating vaccine-derived polioviruses in multiple regions of the world. The Commission noted that the international spread of poliovirus remains a Public Health Emergency of International Concern and emphasized that all countries in the Region must remain vigilant against the risk of importation and subsequent transmission.

The Commission acknowledged the sustained commitment of Member States, National Certification Committees for Polio Eradication (NCCPEs), national immunization programmes, surveillance systems, laboratories and partners in maintaining the Region's polio-free status. The annual reports submitted by all Member States demonstrated continued efforts to sustain population immunity, strengthen surveillance systems, improve outbreak preparedness and response capacities, and implement poliovirus containment activities. The Commission appreciated the progress made by countries in implementing recommendations from previous meetings and endorsed the conclusions and recommendations of the respective NCCPEs.

The Commission noted that regional population immunity remains generally strong, with all countries except Democratic People's Republic of Korea implementing both bOPV and IPV schedules, and all but three countries having introduced two-dose IPV schedules in line with SAGE recommendations. Nevertheless, important immunity gaps continue to persist within and between countries. Particular concern was expressed regarding sub optinsal coverage of IPV2 dose, zero-dose and under-immunized populations, hard-to-reach communities, migrant and displaced populations, and areas with longstanding subnational inequities in vaccination coverage. These immunity gaps increase the risk of poliovirus emergence and transmission if virus importation were to occur.

The Commission reviewed recent detections of variant polioviruses in the Region and noted that all events were detected through sensitive surveillance systems and were followed by timely public health investigations and response measures. The Commission appreciated the rapid actions taken by affected countries and concluded that these events demonstrated the continued value and effectiveness of the Region's surveillance and response capacities. However, the occurrence of these detections also highlighted the continued vulnerability of the Region and the need to sustain high levels of surveillance and population immunity.

The Commission concluded that surveillance systems across the Region remain generally sensitive and continue to perform well at national level. AFP surveillance, environmental surveillance and laboratory networks collectively provide a strong platform for early detection of poliovirus transmission. However, significant operational challenges remain. Subnational variability persists in several countries, and gaps continue to be observed in AFP detection, stool adequacy, timeliness of reporting, specimen transport and case classification.

The Commission recognized the important contribution of environmental surveillance and laboratory networks in the Region. Environmental surveillance now operates in five countries with more than one hundred surveillance sites, while regional laboratories continue to meet high performance standards and provide timely and reliable results. Nevertheless, further optimization and expansion of environmental surveillance based on epidemiological risk will be important to strengthen early detection capabilities and enhance regional preparedness.

The Commission reviewed progress in poliovirus containment and acknowledged that most countries have established mechanisms for inventory management and containment oversight. However, country-specific gaps remain with some countries requiring updated facility inventories,

improved documentation of potentially infectious materials, or strengthened national containment structures.

The Commission also reviewed progress in polio transition and noted that the four transition priority countries in the Region—Bangladesh, India, Myanmar and Nepal—remain at different stages of implementation. While progress has been observed in integrating selected polio functions into broader health systems and government structures, transition milestones remain behind schedule in several countries. The Commission expressed concern that declining external financing, competing public health priorities and inadequate domestic resource mobilization could compromise the sustainability of essential polio functions if transition is not adequately planned and financed.

The Commission recognized that the Region is entering a particularly critical phase of the eradication programme. In addition to risks associated with persistent WPV1 transmission globally and continued cVDPV outbreaks, Member States face challenges related to containment compliance, preparation for the post-eradication era, evolving immunization schedules, and sustainability of polio-funded assets. The Commission emphasized that maintaining a polio-free Region will require continued political commitment, sustained investment in immunization and surveillance systems, effective outbreak preparedness and response mechanisms, and successful integration of essential polio functions into resilient national health systems.

6.2 Overarching recommendations by SEA-RCCPE

The SEA-RCCPE recommended:

- reaffirmation of its previous recommendations to strengthen routine immunization and surveillance performance with particular focus on the subnational level.
- review of the polio vaccine schedule of all countries by respective NITAG's and recommend optimization as appropriate, aligned to SAGE/WHO recommendations.
- feasibility assessment and optimization of environmental surveillance based on epidemiological risk to enhance early detection of poliovirus transmission.
- updating national outbreak preparedness plans and aligning them to SOPs released in June 2026 and regularly conduct outbreak preparedness assessment and simulation exercises.
- conducting regular subnational risk assessments incorporating variables on susceptibility, surveillance, vulnerability and programme factors to guide prioritization of interventions.
- enhancing (inter and intra-regional) cross-border coordination, data sharing, and joint surveillance activities to mitigate importation risks.
- strengthening data systems, including reports from the private sector, to better identify and address programmatic gaps.
- high-level political commitment and long-term domestic financing:
    - to maintain the Region's polio-free status
    - to sustain oversight of poliovirus containment activities through National Authorities for Containment (NACs) and National Containment Task Forces (NCTFs).

- to ensure sustainable and effective polio transition in the four priority countries, with revised milestones and clear timelines to ensure continuity of essential polio functions.

7. Country-specific conclusions and recommendations

The Commission made the following country-specific conclusions and recommendations.

Bangladesh

The SEA-RCCPE noted that the NCCPE is currently undergoing reconstitution; therefore, its functioning was limited during 2025, with only one meeting reported. The Commission appreciated Bangladesh's revision of the IPV schedule in 2026 in accordance with SAGE recommendations. However, it expressed concern about vaccine stockouts at various administrative levels that resulted in suboptimal immunization coverage in several districts. While Bangladesh has continued to sustain its immunization and surveillance systems through substantial partner support such as GAVI HSS3, the Commission noted that progress towards full government ownership of essential polio functions remains incomplete as critical polio functions are heavily dependent on external support and that implementation of polio transition activities continues to lag behind planned milestones.

The SEA-RCCPE recommended:

- strengthening immunization coverage with targeted catch-up strategies focusing on missed children from low-performing districts.
- Ensure age-specific vaccination of children who missed one or more doses during the vaccine stockout period.
- urgent reconstitution of the NCCPE to provide independent oversight to the polio programme and urges the Secretariat to provide briefing to the newly constituted NCCPE.
- urgent development and endorsement of a costed, time-bound transition plan covering human resources, operational funding, commodities, and surveillance/laboratory functions, aligned with existing systems and government ownership.

Bhutan

The SEA-RCCPE appreciated Bhutan for maintaining high immunization coverage for both OPV and IPV and continuing to demonstrate adequate overall surveillance performance. However, the Commission observed that some surveillance indicators, including stool adequacy and timeliness of AFP reporting, require further strengthening. The Commission also noted that the national inventory of laboratories and facilities potentially retaining poliovirus material has not been updated since 2003 and requires attention to ensure continued compliance with containment requirements.

The SEA-RCCPE recommended:

- strengthening AFP surveillance with particular emphasis on stool adequacy and timeliness of reporting.

- to conduct a national survey to identify facilities capable of long-term storage of stools and nasopharyngeal swabs
- Coordination with WHO Regional Office for South-East Asia for feasibility assessment of establishing environmental surveillance.

Democratic People's Republic of Korea

The SEA-RCCPE noted that only a single dose of IPV has been implemented since 2021, resulting in potential vulnerability to poliovirus transmission. The Commission acknowledged that the country has conducted an IPV catch-up campaign and has indicated its intention to introduce a second IPV dose.

The Commission further observed that surveillance systems remain sensitive; however, environmental surveillance has not yet been established in the country. The SEA-RCCPE also noted that information provided in several key programmatic areas, including containment and outbreak preparedness, was insufficient. It further recorded that one facility was reported to retain infectious or potentially infectious poliovirus material. In addition, the Commission noted that subnational risk assessments are not currently being conducted.

The SEA-RCCPE recommended:

- to introduce the second dose of IPV schedules, in line with SAGE/SEAR-ITAG recommendations, urgently.
- a feasibility assessment for conducting environmental surveillance.
- improving reporting completeness across all programmatic areas and providing comprehensive documentation.
- documented destruction of poliovirus infectious or potentially infectious material, stored in one reported facility.
- the Secretariat to urgently conduct onsite accreditation of the National Polio Laboratory.

India

The SEA-RCCPE appreciated India for maintaining high national immunization coverage and a strong surveillance system, including AFP, environmental surveillance and PID surveillance, capable of detecting vaccine-derived polioviruses. The Commission also commended India for conducting national and sub-national supplementary immunization activities in response risk assessment, demonstrating its capacity for effective risk mitigation. However, the Commission cautioned that the ongoing transition from the National Public Health Support Network (NPSN) to the National Centre for Disease Control-Integrated Disease Surveillance Programme (NCDC-IDSP) has introduced variability in surveillance performance and has led to declining performance in several districts. Persistent subnational gaps in immunization coverage also continue to represent an area of concern.

The SEA-RCCPE observed that the nationwide laboratory survey to identify facilities retaining poliovirus infectious and potentially infectious materials was conducted almost ten years ago and that the available information is therefore outdated. The Commission further noted that

containment certificates currently held by the country are no longer valid and that discrepancies remain between the number of designated poliovirus-essential facilities reported nationally and those recorded by the GCC.

The SEA-RCCPE recommended:

- continuing implementation of the recommendations from the 2024 independent EPI and VPD surveillance review, strengthening coordination among partners, and conducting periodic independent surveillance reviews to assess progress and identify areas for further improvement.
- updating list of facilities storing long-term stool and throat swab samples, especially in areas where virus of concern was detected.
- strengthening governance for transition through clear accountability, coordination among NPSN, NCDC-IDSP, and immunization programmes, and regular surveillance review mechanisms during and after transition to address structural transition gaps.
- immediate action, in collaboration with WHO, to apply for valid interim certificates of containment (ICC) or certificates of containment (CC), in compliance with current GAPIV standards, as per published GCC timelines.

Maldives

The SEA-RCCPE appreciated Maldives for maintaining high routine immunization coverage with OPV and IPV and noted the country's plan to introduce hexavalent vaccine from 1 January 2027. However, the Commission observed that surveillance-related challenges persist, particularly in relation to stool adequacy, specimen shipment and timeliness of case classification. The Commission further observed that there is currently no updated national facility inventory.

The SEA-RCCPE recommended:

- that the IPV to Hexavalent transition cohort receives at least 2 dose of IPV.
- strengthening surveillance quality, particularly stool collection, specimen shipment, and case classification timeliness.
- conducting national survey to prepare list of facilities that store human specimens, especially stool and throat swabs for long term.

Myanmar

Myanmar has made considerable efforts to improve immunization coverage through vaccination campaigns and targeted interventions aimed at reaching underserved and high-risk populations. However, despite these efforts, coverage with both bOPV and IPV remains below national targets, with substantial subnational disparities that continue to create pockets of susceptibility to poliovirus transmission.

The SEA-RCCPE expressed concern that chronic surveillance gaps persist, particularly at the subnational level. The Commission noted the continued presence of large number of township as silent areas and delays in specimen shipment, which is affecting the sensitivity and timeliness of poliovirus detection and response. Given the existing large immunity gaps and operational

challenges, the Commission concluded that Myanmar remains at high risk for the emergence of vaccine-derived polioviruses (VDPVs).

The Commission further noted that information provided on polio transition planning was insufficient to adequately assess progress towards the integration and sustainability of essential polio functions. In addition, the SEA-RCCPE observed that Myanmar currently does not have a National Containment Task Force, which remains a critical requirement for strengthening and overseeing poliovirus containment activities.

The SEA-RCCPE recommended:

- prioritizing improvement of immunization coverage, with focus on missed cohorts, high-risk populations, and hard-to-reach areas.
- focused efforts to strengthen surveillance in targeted silent districts and high-risk areas and improve the timeliness of specimen shipment.
- expanding and optimizing environmental surveillance.
- strengthening containment by establishing NCTF and conducting national facility survey.
- developing and implementing a comprehensive, time-bound polio transition plan.

Nepal

The SEA-RCCPE noted that an independent external EPI and VPD surveillance review was conducted in Nepal in 2026 and appreciated the country's continued efforts in risk assessment and outbreak preparedness planning.

The Commission further noted that Nepal has made progress in integrating essential polio functions into broader health systems. However, the SEA-RCCPE concluded that the transition remains incomplete and that the country continues to rely on partner-supported workforce and operational resources to implement essential polio functions.

The SEA-RCCPE recommended fully implementing recommendations from the recent EPI and VPD surveillance review, with priority focus on:

- accelerating endorsement and time-bound operationalization of the transition implementation framework with clear roles, timelines, and accountability across federal, provincial, and municipal levels.
- advocacy with the government for securing sustainable domestic financing and formal budget lines for surveillance, outbreak preparedness, and workforce costs to reduce dependence on external partners.

Sri Lanka

The SEA-RCCPE appreciated the high coverage of polio-containing vaccines achieved at both national and subnational levels. The Commission also noted the country's plan to conduct an external AFP surveillance review in 2027.

The SEA-RCCPE further observed that Sri Lanka is in the process of establishing environmental surveillance and noted that a national survey for poliovirus containment is currently underway.

The SEA-RCCPE recommended sharing the reports of:

- AFP surveillance review as soon as it is available
- establishment of environmental surveillance in the next meeting.
- national facility survey after its completion.

Thailand

The SEA-RCCPE noted the conclusions and recommendations of the independent international EPI and VPD surveillance review conducted in 2025. The Commission appreciated the efforts undertaken to strengthen immunization and surveillance among high-risk areas and populations, including the implementation of supplementary vaccination activities targeting border and other high-risk populations.

Nevertheless, the SEA-RCCPE expressed concern regarding low coverage of the third dose of oral polio vaccine and the persistence of immunity gaps among certain population groups

The SEA-RCCPE recommended:

- timely follow-up on the implementation of the recommendations of the independent external EPI and VPD surveillance review.
- strengthening routine immunization to address immunity gaps, especially among migrants and low-coverage areas.

Timor-Leste

The SEA-RCCPE noted that a joint external EPI and VPD surveillance review was conducted in 2024 and acknowledged the country's efforts to strengthen programme performance.

However, the Commission expressed concern that immunization coverage remains low. The SEA-RCCPE further observed that surveillance performance remains suboptimal, as reflected by low AFP detection rates and the absence of environmental surveillance.

The Commission also noted that containment structures remain inadequate and that there is currently no National Containment Task Force in the country.

The SEA-RCCPE recommended:

- follow-up on the implementation of the joint external EPI-VPD Surveillance review with prioritization on
    - urgently strengthening immunization coverage and reaching zero-dose populations.
    - strengthening surveillance systems and improving AFP detection rates.
- strengthening containment by establishing NCTF and conducting national facility survey.

Annex 1: Agenda

Day 1[1]	Wednesday, 24 June 2026	Speaker/Facilitator
9:00 – 10:00	**Registration**	
9:00 – 10:00	**Pre-meeting of SEA-RCCPE and Secretariat – closed-door session**	
10:00–10:30	**Opening Session**	
10:30–11:00	Group Photo and Tea Break	
11:00-11:50	**Global and Regional updates**	
	Global updates on polio eradication and polio transition (15min) Regional update on maintaining polio-free status and polio transition (20min) Discussion (15 min)	WHO headquarters WHO Regional Office for South-East Asia
11:50–12:50	**NCCPE reports including containment (20 min report + 10 min discussion)**	
	Bangladesh Bhutan	NCCPE
12:50-13:50	Lunch	
13:50–15:20	**NCCPE reports including containment (20 min report + 10 min discussion)**	
	Democratic People's Republic of Korea Maldives Myanmar	NCCPE
15:20 – 15:50	**Tea break**	
15:50 -17:00	**Closed door session (SEA-RCCPE members and Secretariat)**	

[1] The time mentioned in the agenda is Colombo, Sri Lanka time

Day 2	Thursday, 25 June 2026	Speaker/Facilitator
09:00-10:30	**NCCPE reports including containment (20 min report + 10 min discussion)**	
	India Nepal Sri Lanka	NCCPE
10:30-11:00	Tea break	
11:00-12:00	**NCCPE reports including containment (20 min report + 10 min discussion)**	
	Thailand Timor-Leste	NCCPE
12:00–13:00	Lunch	
13:00 – 14:40	**Containment**	
	Global update on containment (20 min)	WHO headquarters
	Regional update on containment (20 min)	WHO Regional Office for South-East Asia
	Country discussion for progress on containment (60 min)	NCTF/NAC, WHO headquarters/WHO Regional Office for South-East Asia and SEA-RCCPE
14:40-15:10	Tea Break	
15:10-17:00	**Closed door session (SEA-RCCPE members and Secretariat)**	

Day 3	Friday 26 June 2026	Speaker/Facilitator
09:00-10:00	**Updates from other Regions (Presentation 10mins + Discussion 5mins)**	
	Eastern Mediterranean Region Western Pacific Region	WHO Regional Office for the Western Pacific WHO Regional Office for the Eastern Mediterranean
10:00-12:00	**Consultation on Polio Transition**	
	Regional Strategic Plan on Polio Transition Group discussion	WHO Regional Office for South-East Asia All participants
11:30-12:00	Working Tea Break	
12:00-13:30	**Concluding session**	
	Conclusions and Recommendations Remarks by Partners Closing remarks	Chair SEA-RCCPE UNICEF, Rotary International, WHO Regional Office for South-East Asia
13:30 -14:30	Lunch	

Annex 2: Message from OIC

Nineteenth Meeting of the SEA Regional Certification Commission for Poliomyelitis Eradication (SEA-RCCPE)

24 June 2026, Colombo, Sri Lanka

Honourable representatives of the Ministry of Health of Sri Lanka, Chair and Members of the South-East Asia Regional Certification Commission for Poliomyelitis Eradication, Chairs and members of National Certification Committees, Representatives of partner agencies, National Containment Focal Points, Colleagues.

Good morning.

I am pleased to address this 19th meeting of the South-East Asia Regional Certification Commission for Poliomyelitis Eradication.

To the newly appointed members of the Commission – welcome.

To the outgoing members – my thanks and appreciation for your dedication and service.

South-East Asia's certification as polio-free in March 2014 was a landmark public health achievement and a testament to what sustained commitment and collective action can accomplish.

Sustaining that achievement has depended on the diligence, technical judgement, and vigilance of this Commission, whose work continues to protect one of the Region's most valuable public health gains.

But polio is not yet history.

Wild poliovirus type 1 continues to circulate in some countries.

Outbreaks of circulating vaccine-derived poliovirus, particularly type 2, have affected multiple regions.

In an interconnected world, any outbreak anywhere can become a threat everywhere.

Immunity and surveillance gaps—particularly among mobile, marginalized and hard-to-reach populations—remain the openings through which this disease moves.

During the course of this meeting, the Commission will review the annual reports and supporting evidence from our Member States.

The guidance you provide will address the key pillars that keep our region polio-free.

First: maintaining high population immunity through routine immunization and targeted strategies for underserved communities.

Second: sustaining sensitive acute flaccid paralysis and environmental surveillance.

Third: strengthening risk assessment.

Fourth: outbreak preparedness—ensuring readiness to respond swiftly and effectively if poliovirus is detected.

Fifth: advancing containment of polioviruses, in line with the Global Action Plan for Poliovirus Containment, fourth edition (GAPIV).

As our Region advances in laboratory strengthening, vaccine production and biomedical research, the number and complexity of facilities handling polioviruses will increase.

The responsibility to prevent a containment breach grows with it.

Finally: Ensuring a smooth and responsible polio transition.

Transition is not an administrative exercise.

It is about protecting what works—core surveillance functions, rapid response capacities, and the laboratory networks that countries have built.

It is about retaining the community engagement platforms, the data systems, and the trained workforce that underpins immunization and outbreak preparedness.

None of this must be lost.

Bangladesh, India, Myanmar and Nepal are the priority countries we have identified where the scale of polio-funded assets and functions is substantial.

Careful planning is essential to avoid any loss of critical capacity in these countries.

We look forward to the Commission's clear conclusions on the Region's certification status, to guidance that is actionable, and recommendations that are prioritized.

These are invaluable for WHO and Member States alike and will ensure that our collective accountability remains strong.

My thanks to the Government and people of Sri Lanka, and the Ministry of Health, for graciously hosting this meeting.

My thanks also to the Chair and members of the Commission for your continued independent oversight; to the National Certification Committees, national containment authorities, and focal points—for the rigour of your work throughout the year.

Together, let us continue to protect every child from poliomyelitis, and keep South-East Asia polio-free.

Thank you.

Annex 3: List of participants

SEA-RCCPE Chair and Members

1. Professor Shrijana Shrestha
   Chair, RCCPE
   Professor of Paediatrics
   Patan Academy of Health Sciences
   Lalitpur, Nepal
2. Professor Jayaprakash Muliyil
   Adjunct Professor
   Centre for Public Health
   Christian Medical College
   Vellore, India
3. Professor Khin Nyo Thein
   Former Professor and Head
   Department of Paediatrics
   University of Medicine 2
   Yangon, Myanmar
4. Dr Sunethra Gunasena
   Medical Virologist, Retired
   Dehiwala, Sri Lanka
5. Dr Pasakorn Akarasewi (virtual)
   Senior Disease Control and Public Health Advisor Bureau of Epidemiology
   Department of Disease Control
   Ministry of Public Health
   Nonthaburi, Thailand
6. Dr Nobuhiko Okabe
   Advisor
   Kawasaki City Institute for Public Health
   Kawasaki, Japan
7. Dr Yagob Yousef Al-Mazrou
   Consultant Epidemiologist
   Former Secretary General
   Saudi Health Council
   Riyadh, Saudi Arabia
8. Mr Christopher Patrick Maher
   Retired Senior Adviser to the
   Director General-WHO
   Brisbane, Australia
9. Dr Arlene King (virtual)
   Adjunct Professor, Dalla Lana School of Public Health, University of Toronto
   Canada
10. Dr Milena Maria Lay Dos Santos
    Pediatric Department, National Hospital
    Guido Valadares bidau
    Dili, Timor Leste
11. Dr Chowdhury Ali Kawser
    Honorary Chief Consultant
    ICDDRB
    Dhaka, Bangladesh

Global Certification Commission (GCC)

12. Professor David Salisbury
    Chair, Global Certification Commission
    Associate Fellow
    Centre on Global Health Security
    Royal Institute for International Affairs
    London, United Kingdom

National Certification Committee for Polio Eradication (NCCPE)

Bangladesh

13. Dr Rajib Sarkar
    Member, NCCPE
    Medical Officer, EPI
    DGHS
    Dhaka, Bangladesh

Bhutan

14. Dr Sangay Thinley
    Chair, NCCPE
    Thimphu, Bhutan
15. Mr Tshewang Dorji Tamang
    Member, NCCPE
    Thimphu, Bhutan

DPR Korea

16. Dr Kwak Song Jun
    Member, NCCPE
    Central Institute for Disease Prevention and Control
    Ministry of Public Health
    Pyongyang, DPR Korea
17. Dr Kim Hyon Jong
    Korea Public Health Cooperation Agency
    Ministry of Public Health
    Pyongyang, DPR Korea

18. Dr Kim Jong Ok
Central Institute for Disease
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Ministry of Public Health
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19. Dr Kim Jong Ran
Central Institute for Disease
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Ministry of Public Health
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20. Dr Ri Ju Bom
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Pyongyang, DPR Korea

21. Dr Ri Sol
National Center for Disease
Prevention and Management
Ministry of Public Health
Pyongyang, DPR Korea

22. Dr Ri Son Hui
Korea Public Health Cooperation Agency
Ministry of Public Health
Pyongyang, DPR Korea

India

23. Professor (Dr) Narendra K Arora
Chair, NCCPE
INCLEN Executive Office
New Delhi, India

Maldives

24. Dr Ali Nazeem
Chair, NCCPE
Internal Medicine, Indira Gandhi Memorial Hospital
Male', Republic of Maldives

Myanmar

25. Professor (Dr) Soe Lwin Nyein
Chair, NCCPE
Director General (Rtd), Advisor & Public Health Expert
Yangon, Myanmar

Nepal

26. Dr Ganesh Rai
Chair, NCCPE
Kathmandu, Nepal

Sri Lanka

27. Dr Palitha Karunapema
Member, NCCPE
Chief Epidemiologist
Epidemiology Unit
Ministry of Health and Mass Media
Colombo, Sri Lanka

Thailand

28. Dr Jurai Wongsawat
Chair, NCCPE
Nonthaburi, Thailand

29. Dr Pawinee Doung-Ngern
Deputy Director, Division of Epidemiology
Department of Disease Control
Ministry of Public Health
Nonthaburi, Thailand

Timor-Leste

30. Dr Maria Angelia Pereira
Head of the Dept-National EPI
Member, NCCPE
Dili, Timor Leste

National Task Force for Polio Containment

Bangladesh

31. Ms Tania Moon
NCTF Representative
Senior Assistant Secretary
Health Services Division
MoHFW
Dhaka, Bangladesh

Bhutan

32. Mr Tashi Dorji
Dy. Chief Laboratory Officer, VPVDL,
RCDC, Ministry of Health
Thimphu, Bhutan

DPR Korea

33. Dr Song Chol Nam
National Center for Disease Prevention and Management
Ministry of Public Health
Pyongyang, DPR Korea

India

34. Dr Labanya Mukhopadhyay
Scientist-C, CD Division,
ICMR HQ
India

Maldives

35. Dr Ismail E Ali
Member, NCCPE
Consultant Paediatrics
ADK Hospital
Malé, Republic of Maldives

Myanmar

36. Dr Win Win Yee (Ms.)
Senior Consultant Microbiologist
National Containment Task Force
National Health Laboratory Yangon
Yangon, Myanmar

Sri Lanka

37. Mrs M A Y Fernando
Senior Lecturer
Department of Microbiology
Faculty of Medicine
Colombo, Sri Lanka

Thailand

38. Mrs Pilailuk Akkapaiboon Okada
(National Containment Task Forces)
Director of National Institute of Health
Department of Medical Sciences
Ministry of Public Health
Nonthaburi, Thailand

Timor Leste

39. Ms Alberina do Carmo Vieira
Senior Clinician Responsible for MR/NHL
Representative from National Containment
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Dili, Timor Leste

Donors and Partners

Rotary International

40. Mr Gulam Husein A. Vahanvaty
National Chair of The National PolioPlus
Committee in India
Rotary International
Mumbai, India

UNICEF

41. Dr Ridwan Gustiana
Health Specialist
Accelerated Disease Control, Immunization,
Centre of Excellence
UNICEF
Bangkok, Thailand

Observers

India

42. Dr Rajib Dasgupta
Professor & Chairperson
Centre of Social Medicine & Community
Health
Concurrent Faculty, School of Sanskrit and
Indic Studies
Jawaharlal Nehru University
New Delhi, India

Sri Lanka

43. Dr Janaki Abeynayake
Consultant Virologist
Head / Virology Laboratory
Medical Research Institute
Ministry of Health and Mass Media
Colombo, Sri Lanka

44. Dr Rasika Gunapala
Consultant Paediatrician
Lady Ridgeway Hospital
Ministry of Health and Mass Media
Colombo, Sri Lanka

45. Dr Sashimali Wickramasinghe
Consultant Epidemiologist
Epidemiology Unit
Ministry of Health and Mass Media
Colombo, Sri Lanka

WHO Country Office

Bangladesh

46. Dr Md Arifur Rahman
NPO (VPD Surveillance)
WCO Bangladesh

Bhutan (virtual)

47. Dr Kencho Wangdi
Health Emergency Support Officer
WCO, Bhutan

48. Dr Lobzang Dorji
    Public Health support Officer (disease control)
    WCO Bhutan

DPR Korea (virtual)

49. Dr Dongbao Yu
    Public health Specialist (epidemiology)
    WCO, DPR Korea

50. Dr Chol Kim
    NPO, WHO Country Office to DPR Korea
    WCO, DPR Korea

India (virtual)

51. Dr Arun Kumar
    Zonal Coordinator- NTD
    WCO, India

52. Dr Ratnesh Murugan
    NPO (Measles and Rubella Initiative)
    WCO, India

Maldives (virtual)

53. Ms Sarah Jamal
    NPO (Communicable Diseases and Environment)
    WCO, Maldives

Myanmar (virtual)

54. Dr Raveesha Mugali
    Medical Officer (EPI & VPD Surveillance)
    WCO Myanmar

55. Dr Khaing Khaing Gyi
    National Professional Officer (Immunization & Surveillance)
    WCO Myanmar

Nepal

56. Dr Pasang Rai
    NPO (VPD Surveillance)
    WCO Nepal

Sri Lanka

57. Dr Farrukh Qureshi
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    WHO Country Office, Sri Lanka

58. Dr Dushan Sameera Hewage
    NPO (Communicable Disease Control)
    WHO Country Office, Sri Lanka

Timor Leste (virtual)

59. Dr Nazmun Nahar
    Technical Officer (Expanded Programme on Immunization)
    WCO Timor Leste

60. Dr Mateus Cunha
    National Professional Officer
    WCO Timor Leste

61. Dr Dulce Magno Pereira
    NPO (Immunization)
    WCO Timor Leste

Headquarters

62. Dr Mufti Zubair Wadood
    Team Lead
    WHO/HQ

63. Dr Liliane Boualam (virtual)
    Technical Officer
    WHO/HQ

Regional Office for the Eastern Mediterranean

64. Dr Ashraf Wahdan (virtual)
    Medical Officer, EMRO Polio Eradication Programme & Vaccine – Prevention Diseases
    WHO-EMRO

Regional Office for the Western Pacific

65. Dr Syeda Kanwal Aslam
    Technical Officer - DPC
    WHO-WPRO

WHO Secretariat

Immunization and Vaccines Development (IVD), WHO-SEARO Secretariat, New Delhi, India

66. Dr Vinod Kumar Bura
    Coordinator, IVD

67. Dr Sudhir Khanal
    Regional Adviser (Accelerated Disease Control), IVD

68. Dr Sudhir Joshi
    Technical Officer (Polio Endgame), IVD

69. Dr Lucky Sangal
    Virologist/immunologist, IVD

70. Ms Pooja Mehrotra
    Executive Assistant, IVD



The WHO South-East Asia Regional Certification Commission for Poliomyelitis Eradication (SEA-RCCPE) provides independent oversight to the polio eradication programme in the South-East Asia Region.

The Commission meets annually to review reports submitted by the National Certification Committees for Poliomyelitis Eradication (NCCPEs) of Member States, and to assess progress in maintaining wild poliovirus-free status, monitoring vaccine-derived polioviruses, advancing poliovirus laboratory containment, and implementing polio transition activities.

This report presents the conclusions and recommendations of the SEA-RCCPE from its nineteenth meeting held in June 2026. The Commission concluded that the WHO South-East Asia Region maintains its polio-free status and that there is no evidence of wild poliovirus (WPV) circulation in the Region.